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A. Purpose: To provide guideline instruction for management of pain.

B. Scope : Hospital wide

C. Policy : The hospital and its staff members:

• Recognizes the right of individuals to appropriate assessment and management of pain.

• Plans, supports, and coordinates activities and resources to ensure that the pain of all individuals is recognized and addressed appropriately.

• Provides individualized care in settings responsive to specific needs.

• Provides education on pain management as part of the patient's treatment considering the patient's personal, cultural, spiritual, and/or ethnic beliefs.

• Develops plan in conjunction with the patient, if on discharge the patient has pain, to address management at home.

• Monitors the performance of the pain management program.

D. Patient Rights

1.Patients rights include :

a. Information about pain and pain relief measures,
b. A concerned staff committed to pain prevention and management,
c. Health professionals who respond quickly to reports of pain,
d. That reports of pain will be believed,
e. State-of-the-art pain management.

2. Patients responsibilities which include:

a. Ask the doctor or nurse what to expect regarding pain and pain management,
b. Discuss pain relief options with the doctors and nurses,
c. Work with the doctor and nurse to develop a pain management plan,
d. Ask for pain relief when pain first begins,

e. Help the doctor and nurse assess the pain,
f. Tell the doctor or nurse if the pain is not relieved, and
g. Tell the doctor or nurse about any worries regarding taking pain medication.

E. Assessment:

1. A patient's report of pain will be accepted and respected as the key indicator of the amount of pain he/she is experiencing. Medical/nursing staff will assign the rating only if the patient is unable to report their pain.

2. The presence of pain is assessed on admission to the hospital, at the initial clinic visit, post invasive procedure and when the patient complains of pain. The assessment is performed by: a physician,anaesthetist, nurse and documented in the medical record.

3. The frequency of pain reassessment shall be dictated by the intensity of the patient's pain and the effectiveness of pain relief strategies. However, when pain is present, a pain reassessment is generally performed at least every 4 hours and more often as needed by a licensed healthcare provider. The physician is notified of the patient's pain when treatment fails to reduce the pain to a level acceptable to the patient, as ordered by the physician, or pain score > 5 using the approved Pain Scales. If no pain is present, the healthcare provider will reassess for pain as warranted by patient condition, when the patient complains of pain and post invasive procedure.

4. Pain Scales :

a. The Numeric Pain Intensity Scale (NPIS) will be used universally to assess pain for patients 13 years or older. Patients will be asked to rate their pain a scale of 0-10. Zero represents no pain; a rating of 5 would indicate that the patient is experiencing moderate pain, and a rating of 10 would indicate the worst imaginable pain.

b. The Wong-Baker Faces Pain Scale, consisting of graduated facial expressions of pain, will be used for patients, ages < 13 and those unable to comprehend the numerical scale. Zero will represent no hurt and a rating of 10 would indicate the patient is experiencing the worst possible hurt.

5. If pain is present, a more comprehensive assessment is performed, which may include:

a. Intensity (Numerical 0 -10, Wong-Baker Face Scale)
b. Quality
c. Location(s) (All pain locations are assessed)
d. Onset
e. Duration
f. Variation
g. Alleviating and aggravating factors
h. Present pain management regimen and effectiveness
i. Medication history
j. Presence of common barriers to reporting pain and using analgesics
k. Past interventions and response
l. Manner of expressing pain
m. Effect of pain on activities of daily living, sleep, appetite, relationships, emotions and concentration.
n. Pain goal, expressed as measures of intensity and function.
o. Physical examination:

1) Mental status examination
2) Motor and sensory examination
3) Reflexes
4) Gait
5) Maneuvers targeted to pain diagnoses
6. Documentation of pain, for all patients, should include the following:

a. Type of pain and/or location
b. Intensity scale
c. Level of consciousness
d. Respiratory rate
e. Activity
f. Side effects
g. Medication
h. Patient and family education
i. Treatment goal.

7. Staff shall be educated about pain assessment, including the availability of nonpharmacological interventions.

F. Treatment

1. Pain is managed by pharmacological treatment, nonpharmacological treatment, and interventional procedures.

a. Pharmacological treatment may include non-opioids, opioids, and adjuvants.

b. Non pharmacological treatment may include physical interventions and cognitive behavioral strategies.

1) Physical interventions may include:

a. Heat
b. Cold
c. Electrical stimulation (e.g TENS)
d. Exercise
e. Physical/Occupational therapy
f. Immobilization
g. Manipulation
h. Massage
i. Acupuncture

2) Cognitive behavioral strategies may include:

a). Distraction
b). Relaxation
c). Hypnosis
d). Other coping strategies

2. The hospital provides safe medication prescription or ordering.

a. Pain medication shall be ordered to be given as a specific dose with a regular schedule.

b. PRN orders shall include specific indications for specific dosing. Example: Time ranges such as “every 2-3 hours prn ” are not acceptable. A specified interval such as “every 3 hours prn pain” is acceptable.

c. Range orders shall be avoided unless accompanied by a sliding scale. Example: Dose ranges such as 4-10 mg. Morphine IV every 3 hours are not acceptable unless it is tied

to a measurable pain severity measure (i.e. For pain rating 5-7 administer morphine 5 mg. IVP every 2 hours prn pain; For pain rating 8-10 administer morphine 10 mg IVP every 2 hours prn pain).

d. Specific protocols shall be used for PCA and epidural analgesia.

e. Only one long-acting agent shall be prescribed at any time.

G. Pain Reassessment:

A. Pain will be reassessed:

1. Every eight hours for all hospitalized patients.

2. For patients will high potential for pain (i.e. post surgical patients, patients with chronic pain).
   a. at least every 2 hours for the first 24 hours, then every 4 hours.
   b. prior to pan relieving intervention.
   c. within 30 – 60 minutes after pain relief intervention.

H. Patient Education

1. Patient education may focus on fears commonly held by patients in pain, including:
   a. Fear of drug addiction,
   b. Fear of drug dependence,
   c. Fear of drug tolerance,
   d. Fear of appearing uninformed or unable to understand, and
   e. Fear of inability to function normally.

Patient teaching will include the following:

1. Patient and family education about physiological causes of pain that might be specific to patient (e.g., mass pressing on nerve, tumor obstructing bowel, etc.).

2. Patient and family education about some non-invasive methods, which might help prevent or alleviate pain (e.g., distraction, music, reading, prayer, meditation, guided imagery, massage therapy, etc.).

I. Pain Scale:

1) Numerical pain scale (0-10)
2) Wong-Baker Faces pain scale (0-10)

c. Explanation of treatments:

1) Pharmacological
2) Procedural
3) Non-pharmacological

J. Discharge:

Discharge notes shall include reference to physical needs, emotional needs, and symptom management.

K. Documentation:

1. Inpatient: The following information will be documented

a. a .initial pain screening.
b. initial pan assessment
c. pain score .
d. pain management procedure used.
e. discharge instructions

2. Outpatient: The following information will be documented

a. initial pain assessment.
b. initial pain screening
c. pain score.
d. pain management procedure
e. instructions for the patients.